

Transthoracic Echocardiogram Report

Report Date:	2025-12-05
Cardiologist:	Dr. Mohamed Ben Ali, MD
Patient Name:	Omar Dhaouadi
Patient ID:	19874562
Date of Birth:	15 March 1987
Age:	38 years
Gender:	Male
Height:	178 cm
Weight:	92 kg
BSA:	2.1 m ²
BMI:	29.0 kg/m ²
Examination Date:	2025-12-05
Indication:	Evaluation of exertional dyspnea and occasional palpitations for 3 months
Echocardiograph:	Philips Epiq 7C, Adult cardiac transducer

Findings

Left Ventricle

- Size: Mildly increased (LVEDD 5.7 cm, indexed 2.7 cm/m²; normal <5.5 cm / <2.8 cm/m²).
- Wall thickness: Septum 1.2 cm, posterior wall 1.1 cm (concentric hypertrophy).
- Systolic function: Global longitudinal strain -17% (mildly reduced). Ejection fraction (biplane Simpson) 52% (mildly reduced).
- Regional wall motion: Hypokinesia of mid and apical anteroseptal segments.
- Diastolic function: Grade I diastolic dysfunction (impaired relaxation pattern). E/A ratio 0.8, average E/e' 12.

Right Ventricle

- Size and function normal. TAPSE 22 mm, S' wave 13 cm/s.

Atria

- Left atrium: Mildly enlarged, volume indexed 38 mL/m² (normal <34 mL/m²).
- Right atrium: Normal.

Valves

- Mitral valve: Structurally normal. Trace physiological regurgitation.
- Aortic valve: Tricuspid, mild sclerosis without stenosis. Trace regurgitation.
- Tricuspid valve: Normal. Mild physiological regurgitation. Estimated PASP 28 mmHg + RAP (normal).
- Pulmonic valve: Normal.

Pericardium / Other

- No pericardial effusion.
- Aorta: Proximal ascending aorta 3.4 cm (normal).
- Inferior vena cava: Normal size and respiratory collapse.

Measurements Summary

LV Ejection Fraction (Simpson)	52%
LV Mass Indexed	118 g/m ² (elevated)
LA Volume Indexed	38 mL/m ² (mildly elevated)
E/A Ratio	0.8
Average E/e'	12
Global Longitudinal Strain	-17%

Impression

1. Mildly reduced left ventricular systolic function (EF 52%) with mild concentric hypertrophy and regional wall motion abnormality (mid-anteroseptal hypokinesia).
2. Grade I diastolic dysfunction with mild left atrial enlargement.
3. Likely secondary to long-standing hypertension with possible early ischemic component.
4. No significant valvular disease.

Recommendations

- Optimize antihypertensive therapy (target BP <130/80 mmHg). Consider adding ACE inhibitor/ARB if not already on.
- Cardiac risk factor modification: weight loss, regular aerobic exercise, low-salt diet.
- Consider stress testing or coronary CT angiography to evaluate for coronary artery disease given regional wall motion abnormality.
- Repeat echocardiogram in 12 months or sooner if symptoms worsen.
- Cardiology follow-up in 4–6 weeks.

Cardiologist:

Dr. Mohamed Ben Ali, MD

Consultant Cardiologist

Electronically signed: 2025-12-05